Brigatinib monotherapy · ALK-positive advanced NSCLC Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NCCP 00562 · VERSION 4

Brigatinib Therapy

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Lung inflammation or early breathing reactions, which can be serious or fatal

☐ Liver injury and abnormal liver tests

☐ Raised muscle enzyme with muscle pain or weakness

☐ Visual disturbance

☐ Skin rash and sensitivity to sunlight

☐ Allergic reactions; pregnancy and fertility risks

☐ Raised blood pressure or a slow heartbeat

☐ High blood glucose

☐ Raised pancreatic enzymes; report severe abdominal pain

☐ Diarrhoea, nausea, vomiting, reduced appetite and fatigue

☐ Low blood counts and infection risk

Discuss the exact indication and prior ALK treatment, likely benefit and uncertainty, alternatives including not proceeding, serious lung/heart/liver/glucose/vision risks, common symptoms, fertility and contraception, monitoring and the practical tablet plan. Agree who reviews a hold or restart. Consent can be reconsidered.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: ALK result, blood count, kidney/liver tests, breathing-function assessment, ECG, blood pressure, fasting glucose, muscle enzyme CPK and pancreatic enzymes. Review breathing symptoms on days 8 and 15, then monthly or sooner if needed. ECG and blood pressure are checked every two weeks initially, then monthly. Liver tests are every two weeks for the first three months, then as advised. Blood count, kidneys, fasting glucose, CPK and pancreatic enzymes are usually checked monthly. Scans follow your Consultant’s plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending